Pearson

Examiners' Report
Summer 2026

GCSE History 1HI0 11

ResultsPlus

Edexcel and BTEC Qualifications

Edexcel and BTEC qualifications come from Pearson, the UK's largest awarding body. We provide a wide range of qualifications including academic, vocational, occupational and specific programmes for employers. For further information visit our qualifications websites at https://www.edexcel.com or https://www.btec.co.uk.

Alternatively, you can get in touch with us using the details on our contact us page.

ResultsPlus

Giving you insight to inform next steps

ResultsPlus is Pearson's free online service giving instant and detailed analysis of your students' exam results.

- See students' scores for every exam question.
- Understand how your students' performance compares with class and national averages.
- Identify potential topics, skills and types of question where students may need to develop their learning further.

For more information on ResultsPlus, or to log in, visit https://www.edexcel.com/resultsplus. Your exams officer will be able to set up your ResultsPlus account in minutes via Edexcel Online.

Pearson: helping people progress, everywhere

Pearson aspires to be the world's leading learning company. Our aim is to help everyone progress in their lives through education. We believe in every kind of learning, for all kinds of people, wherever they are in the world. We've been involved in education for over 150 years, and by working across 70 countries, in 100 languages, we have built an international reputation for our commitment to high standards and raising achievement through innovations in education. Find out more about how we can help you and your students at: https://www.pearson.com/uk.

Summer 2026

Publications Code 1HI0 11

All the material in this publication is copyright

© Pearson Education Ltd 2026

Introduction

Candidates had clearly been well-prepared for this examination in terms of both knowledge of content and the skills required for this paper. Examiners noted many answers showed confident treatment of answers in both sections, the Historic Environment and the Thematic Study.

Many of the following comments are made every year and remain relevant. As a general point, centres should remember that the Thematic Study focuses on change and continuity over time and therefore a good sense of chronology is vital. Candidates should be familiar with the names given to the different periods in the specification and recognise the dates and key events involved in these chronological divisions. They also need a clear understanding of the key themes and the factors involved in the Thematic Study, as identified in the specification.

· Ideas about the cause of disease and illness.

· Approaches to prevention and treatment.

· Individuals and institutions (Church and government), science and technology, and attitudes in society.

It is also important to remember that this is a Thematic Study in British history. Examples from other countries cannot be rewarded.

In the extended answers, the stimulus points are usually intended to remind candidates to cover different aspects of content and the full timescale of the question. Candidates do not need to include these stimulus points in their answer, but they do need to cover three aspects of content in order to show breadth in their answer and to access the higher marks.

A number of answers to these questions remained at Level 3, despite excellent knowledge, because they missed the focus of the question. The mark scheme's bullet point for Assessment Objective 2 (analysis) at Level 4 expects an analytical explanation, directed consistently at the conceptual focus of the question. Candidates who responded to the topic rather than the key idea were unlikely to achieve high marks. Those who did reach Level 4 realised that the topic provides the context but that there is a specific focus, which the analysis should address.

While the target for the 12–mark question is an explanation of causation, there is no expectation that causes will be prioritised or evaluated and no marks are available for such comments. However, in the 16–mark questions there is an additional element of judgement. Many candidates structured their answers in questions 5 and 6, to discuss points supporting the statement in the question, then points challenging the statement, before offering their judgement. In a number of answers, this resulted in a judgement that summed up the two sides, with the conclusion that the statement was 'somewhat true' or 'true to an extent'. This is a logical structure and can be very effective but for the higher marks, the criteria being applied need to be explained and the judgement needs to be consistent with the overall answer. In high scoring answers, the application of appropriate criteria included an explanation that some aspects had a longer lasting impact, more people were affected, a factor acted as a catalyst for other developments, for example, and many high-scoring answers had a sense of evaluation running throughout the answer so that judgement was not just restricted to comments at the start and end of the answer.

GCSE History specification and assessment changes

This note is a reminder that minor changes are being made to the Pearson Edexcel GCSE History specification content and assessment model: assessment and language changes apply from the June 2025 exam series onwards, and content changes from the June 2026 exam series onwards. We strongly recommend, if you haven't already, that you familiarise yourself with these changes in time for the start of the new academic year in September 2025. Some options are affected more than others, but all options are affected by the assessment changes. Below is a link to the summary guidance document, which will help you to find all the specific information and guidance available to help you take on board these changes:

https://qualifications.pearson.com/content/dam/pdf/GCSE/History/2016/Teaching-and-learning-materials/gcse-history-changes-for-2025–and-2026–summary-guidance.pdf

If you have any queries, please contact Mark Battye, our History subject advisor, at TeachingHistory@pearson.com.

Question 1

This is a two-part question, with a question asking for a key feature with some additional supporting knowledge of two aspects of content in the specification. Extended knowledge is not needed but it is important that candidates know something about all aspects of the specification. This is so that they can add a further detail which will explain the feature or provide some context. They should make sure that the additional detail provided is linked to the key feature that has been identified.

Question 1a was about the Casualty Clearing Station (CCS). Examiners felt that most candidates were able to correctly identify a feature of the Casualty Clearing Station and supported this with relevant information. A wide range of features were provided by candidates. The most common features identified were role of the CCS in triage, the location of the CCS, as well as the CCS being far back from the front line enabling a range of treatments to take place, including surgery. Most students were able to support their feature with a range of details such as the CCS being in a place of safety, the operating theatres, mobile X-ray machines and the work of RAMC doctors and nurses.

Several candidates were only able to provide a basic understanding of the CCS as a place where wounded soldiers received treatment, without providing additional supporting information. There were also several candidates who did not understand the function of the Casualty Clearing Station and incorrectly placed the CCS close to the frontline.

Question 1b was about the developments in the storage of blood on the Western Front. Examiners felt that this question was answered well by candidates and in some cases better than Question 1a. Many candidates were able to provide precise knowledge on the use of sodium citrate to stop the blood clotting and citrate glucose which allowed blood to be stored in refrigerated conditions. There were also multiple references to the first blood bank before the Battle of Cambrai in 1917. Several candidates drifted from the question and described blood transfusions rather than focusing on how developments improved the storage and preservation of blood, making blood transfusion more accessible

Candidates are reminded to use the mark and the space in the answer booklet as a guide for the length of their answer. There were relatively few answers that continued beyond the lined space, but these were often wasting time as the answer had already scored full marks, and no further marks could be awarded. Also, answers which listed two disconnected points of information were limited to a maximum of one mark.

1 (a) Describe **one** feature of a Casualty Clearing Station (CCS).

(2)

they would clean up your injuries. If you got wounded.

(b) Describe **one** feature of developments in the storage of blood on the Western Front.

(2)

One feature of the developments in the storage of blood on the Western Front. they would keep blood stored/ frozz for people who need spear blood.

ResultsPlus

Examiner Comments

The answer to question 1a is 1 mark because it is a valid feature of the work done at the CCS. The answer to question 1b is 1 mark because a valid feature has been provided (froze). In both cases additional support is not given.

ResultsPlus

Examiner Tip

Once you have written a feature, add some additional supporting knowledge about that feature for the second mark.

1 (a) Describe **one** feature of a Casualty Clearing Station (CCS).

(2)

They were located far away from the front lines in order to reduce the risk of enemy attacks while treating wounded Soldiers. This was done in a triage system to seperate the men into there injury levels and keep them safe from danger.

(b) Describe **one** feature of developments in the storage of blood on the Western Front.

(2)

one feature of the storage of blood was that it was stored using Sodium Sitrate. This was to stop the blood from clotting in the glass jars and to allow it to stay usable for a longer period of time.

ResultsPlus

Examiner Comments

For both answers the candidate has provided a valid feature and some additional information about the features. Therefore, both part a and part b were awarded 2 marks each.

ResultsPlus

Examiner Tip

Once you have written a feature and some additional supporting information, you can stop and move on. On Question 1a the candidate has added information about the triage system. This is correct but the candidate has already achieved the two marks for the first part of their answer.

Question 2(a)

The Historic Environment has a focus on the process of history, considering the value of sources as evidence and the way an historian follows up an enquiry. It is nested within the context of the Thematic Study and therefore knowledge of both the specific and the general context is expected.

It is important to note that the question asks about the usefulness of the sources in relation to a specific enquiry, in this case an enquiry into the problems of transporting the wounded along the chain of evacuation. An important reminder is that the focus should be on assessing the usefulness of what is in the source rather than listing details which are not mentioned. A source should not be dismissed because it does not cover every detail that might be helpful in an investigation. Candidates should also recognise that unreliable sources can be very useful.

The question asks 'how useful' the sources are, so a judgement should be made on the usefulness of the source's evidence for the specific enquiry. Examiners felt that both sources were accessible, and candidates were confident in showing that the content of the sources was relevant for the enquiry and were therefore useful. Examiners noted most candidates attempted to analyse both sources. They noted that at Level 2, developed comments were made about the usefulness of the content, for example on Source A many candidates focused on the lack of stretcher bearers suggesting that injured soldiers would have to wait for transport. They also suggested that the terrain ('waterlogged shell holes') the stretcher bearer had to navigate could make the injuries of the wounded soldiers worse. Similarly on Source B candidates suggested that the muddy terrain prevented the motor ambulance from quickly and effective evacuating the wounded. Contextual knowledge was added to support the inferences being made, such as the distance stretcher bearers had to cover to get the wounded to the Regimental Aid Post or Dressing Station, the number of people required to carry a stretcher and high number of casualties that were sustained at the Battle of the Somme. Many candidates were well informed about the horse-ambulances that were an alternative to motor ambulances linked to Source B.

Some very good Level 2 answers could not access the higher marks because they did not include contextual knowledge. Contextual knowledge is mentioned at every level of the mark scheme and failure to include it impacted a number of otherwise good answers. Candidates should also recognise that it is not enough to repeat a detail from the source and assert that this can be confirmed from the candidate's own knowledge – some additional detail is needed as a demonstration of that own knowledge. Candidates are not required to write a large amount of contextual knowledge. They only need to provide contextual knowledge to help assess the usefulness of the source content and/or provenance.

At Level 3, candidates need to show the effect of the provenance on the usefulness of the source content, for example candidates understood that the high level of casualties described by Edward Munro was accurate when linked to their contextual knowledge about the Battle of the Somme. Many went on to explain that as the source was written in a diary, and not written for publication, he was able to describe accurately the casualties that needed to be rescued. For Source B, candidates were able to explain that the photograph, although accurate, might have been used as propaganda to improve morale because people at home could see the efforts soldiers were putting in to evacuate the wounded. Good Level 3 answers made clear the criteria being used to assess the

usefulness of the sources for the enquiry, for example accuracy of detail, reliability and typicality of the source content.

There is no requirement to compare the sources or to use them in combination and no marks are available for this. Candidates who focused on comparisons between the sources often failed to develop their judgement on each source properly; if this approach is used, it is important that the answer still comes to a judgement on each individual source. Very few answers only considered one source, but it should be noted that every level of the mark scheme refers to 'sources' and therefore answers which do not consider both sources cannot access high marks.

How useful are Sources A and B for an enquiry into the problems of transporting the wounded along the chain of evacuation on the Western Front?

Explain your answer, using Sources A and B and your knowledge of the historical context.

(8)

Source A is very useful for an enquiry into the problems of transporting the wounded along the chain of evacuation on the Western Front because it is a reliable source that is first hand from someone who worked ~~on the~~ as a stretcher bearer. From this diary entry, we can see that there wasn't 'enough stretcher bearers' ~~not~~ meaning the ones they did have were rushed to work fast in order to get to everyone. Another thing we are told is that the ground had "many water logged shell holes". This meant that the conditions on the front were poor, making it rather difficult for the stretcher bearers to do their jobs.

Source B is also a reliable source because it is a photograph and you couldn't have photoshoped or tampered with it. However, it could have been staged. In this source we see that it took several men to move this one ambulance. We can also see how they

are using wooden planks in order to get the ambulance up a mound of mud. This means that the conditions were so poor that it took several men to move only 1 ambulance. Therefor both sources are some what reliable, but Source A is better as it is a personal diary.

ResultsPlus

Examiner Comments

Both Source A & B have a developed comment on the usefulness of the source content. The comment on the source provenance is underdeveloped. Neither source has any contextual knowledge. Overall, this a mid-Level 2 response.

ResultsPlus

Examiner Tip

Add some contextual knowledge to help assess the usefulness of the source content and or provenance.

Content
prevenance → TAP Tone Author Purpose
context

(a) **Study Sources A and B in the Sources Booklet.**

How useful are Sources A and B for an enquiry into the problems of transporting the wounded along the chain of evacuation on the Western Front?

Explain your answer, using Sources A and B and your knowledge of the historical context.

(8)

Source A presents transporting the wounded along the chain of evacuation as problematic because of the hazards in the area that made it difficult to get through. Source A mentions a lack of Stretcher bearers and 'many casualties' caused by the 'very difficult to cross' no man's land. This shows the problems of transporting the wounded as it shows that even to get to first aid at the Regimental Aid post, the ~~Stretcher~~ stretcher-bearers, who are in the middle of an active war site, shown by them being constantly shelled by the ~~German~~ Germans, and that '2 stretcher bearers had been killed', ~~showing the dangers and problems~~ have to walk through these 'indescribed shell holes' with a heavy load ~~on their~~ of a wounded soldier. The ~~author~~ ~~of the~~ source being ~~from~~ a diary of a Stretcher bearer ~~demonstrates~~ at the time of war (1916) proves it's ~~from~~ prevenance because a diary would usually hold an accurate accounts of ~~a~~ Stretcher ~~Evidence to support this~~. My own knowledge supports the source because there were many casualties

because of the first day (July 1st 1916) of the battle of the somme, where 20,000 soldiers died, and 50,000 were wounded, which meant a huge amount of specialised staff such as stretcher bearers was needed

Source B presents transporting the wounded along the theme of evacuation as problematic through the picture of many soldiers pushing a motor ambulance during the Battle of the Somme. two soldiers can be seen in front of the ambulance, placing down a plank of wood, possibly for the ambulance to drive on over the mud that it looks to be stuck in. The fact that this was published in a Newspaper as an official war photograph, could imply that the photo might have been used for propaganda to entice others to join the Army, ~~however~~ increased by the huge amount of casualties that the Battle of the Somme, when the image is set, caused, which could showing that they needed more soldiers. However, my own knowledge confirms that the ~~mud~~ trenches and especially no men's land were extremely muddy and waterlogged due to it once being used as agricultural ~~land~~ land before the war, which makes the photo look more credible because the mud would make it a challenge to get to the RAP as mentioned.

ResultsPlus

Examiner Comments

The answer to both sources provides a judgement of the source's utility based on how the provenance affects the usefulness of the content and contextual knowledge is used in the process of interpreting the source. This is a high Level 3 response.

ResultsPlus

Examiner Tip

Use a criterion for usefulness, such as credible, typical or accurate as valid criteria for judgement.

Question 2(b)

This question should be treated as a package linked to the enquiry that was identified in question 2a (problems of transporting the wounded along the chain of evacuation) and the aim is for candidates to show that they understand how historians work. The first sub-question simply asks candidates to identify a detail from the source. This was most commonly done by quoting a phrase from the source. The most common details were "Yesterday, two stretcher bearers had been killed", or "Carrying the wounded was very difficult at night", or the terrain had "many waterlogged shell holes".

Candidates then had to propose a question they would ask to follow up Source A in relation to the overall enquiry. Consequently, the proposed question should be broader than following up a very specific person or event in the source and it should not be a question they would ask the author of the source. Questions about how many stretcher bearers were killed or how the stretcher bearers overcame the problems of the terrain were some of the more common questions to be proposed by candidates.

However, there were a number of candidates who failed to recognise the link with the broader enquiry of the problems of transporting the wounded along the chain of evacuation. This led to candidates proposing questions such as how many soldiers died during Battle of the Somme? This failure to recognise the link to the broader enquiry impacted the marks available to these candidates for this question.

While it is recognised that candidates cannot have detailed knowledge of all possible sources, the specification states that candidates should be aware of the types of sources available and the nature of the information they contain. Answers such as 'medical records' or 'diaries' are too generalised to be rewarded. In some cases, where a generalised source was named in sub-question three, a mark could be awarded because the explanation in the final sub-question made it clear what sort of information might be located in those records and how that information would help the historian with the overall enquiry. Examiners **again** noted that candidates were rote learning RAMC medical records as the source for sub-question three, however this source is not always relevant and will not always contain the information that would help answer the proposed question. For example, RAMC medical records might not answer the question about how typical the difficult terrain was across the Western Front. A more relevant source for this question might be diaries from other stretcher bearer across the Western Front throughout WW1.

Candidates should be showing an awareness of appropriate sources that already exist for the historian to consult. This means that answers suggesting they would carry out an interview were not rewarded. They also need to be clear that they should suggest a contemporary source of the period in question – history books, the Internet, documentaries were all unsuitable answers. Instead, it would be more appropriate if they tried to think about the sources consulted by the producers of history books, Internet articles or documentaries.

When multiple suggestions had been given to a sub-question, it was often counter-productive. Offering more than one detail or question meant that the follow-up sections were not clearly linked, while offering multiple sources meant that the explanation in the final section was usually invalid.

(b) **Study Source A.**

How could you follow up Source A to find out more about the problems of transporting the wounded along the chain of evacuation on the Western Front?

In your answer, you must give the question you would ask and the type of source you could use.

Complete the table below.

(4)

Detail in Source A that I would follow up:	"We started to carry the wounded and there were a large number of them"
Question I would ask:	~~how many wounded soldiers were there?~~ What wounds and injuries did you deal with?
What type of source I could use:	Army medical records from the the battle of the Somme on the western front.
How this might help answer my question:	See what injuries soldiers experienced in war.

ResultsPlus

Examiner Comments

A valid detail has been selected from Source A. The question proposed does not explicitly link to the wider enquiry about the problems of transporting the wounded along the chain of evacuation and is therefore not rewardable. The sources proposed in sub-section 3 and the explanation in sub-section 4 all link to the question which is not valid. Therefore, sub-section 3 and 4 are not rewardable.
Overall, only sub-section 1 is valid, and this answer received a score of 1 mark.

ResultsPlus

Examiner Tip

Make sure the follow up question is related to the wider enquiry of the question.

(b) **Study Source A.**

How could you follow up Source A to find out more about the problems of transporting the wounded along the chain of evacuation on the Western Front?

In your answer, you must give the question you would ask and the type of source you could use.

Complete the table below.

(4)

Detail in Source A that I would follow up: "The land over which we carried the wounded is very difficult to cross"
Question I would ask: Did the lands condition effect the speed ~~the~~ and time it took for soldiers to be seen & impact fatalities?
What type of source I could use: Medical records from the Battle of the Somme 1916
How this might help answer my question: It will help see if the bad terrain played a role in how many soldiers died due to treatable injuries

ResultsPlus Examiner Comments

A valid detail has been selected from Source A. The question proposed is linked to both the wider enquiry and is linked to the detail selected. Therefore, the question is valid. The source identified in sub-section 3 would not provide the answer to the question proposed and is therefore not rewardable. The explanation does not explain how the source would answer the question proposed so is also not rewardable. Therefore, only sub-sections 1 and 2 were rewarded and the answer received a score of 2 marks.

ResultsPlus Examiner Tip

Make sure that the source that you identify will reasonably contain the information that you want to find out about.

(b) **Study Source A.**

How could you follow up Source A to find out more about the problems of transporting the wounded along the chain of evacuation on the Western Front?

In your answer, you must give the question you would ask and the type of source you could use.

Complete the table below.

(4)

Detail in Source A that I would follow up:

"carrying the wounded is very difficult at night. The landmarks which guide us during the daytime are not visible at night meaning it is easy to get lost."

Question I would ask:

what methods did stretcher bearers use to make navigating their way back at night easier?

What type of source I could use:

A diary of a stretcher bearer during world war one about nighttime navigation.

How this might help answer my question:

~~A selfie~~ the diary extract may explain how they managed to find their way back during the night.

ResultsPlus

Examiner Comments

A detail has been selected from Source A. The question proposed is linked to both the wider enquiry and to the detail picked out in sub-section 1. It is reasonable to suggest that a diary from a stretcher bearer during World War One will explain how they managed to navigate their way during the night. Therefore, the source will provide an answer to the proposed question. Both sub-section 3 and 4 are valid. Overall, all sub-sections are valid, and the answer received 4 marks.

ResultsPlus

Examiner Tip

In sub-section 4 remember to clearly explain what is likely to be written in your chosen source and how that information will help to answer your proposed question.

Question 3

It is important that candidates note whether the focus of this question is to identify a similarity or a difference, the topic and the periods in the question. The best answers started with a statement of the overall difference and then provided details from each period to support that comment. Details of the events or periods by themselves cannot score highly as such an answer is not meeting the AO2 strand of the mark scheme covering the analysis of the difference. Alternatively, answers that gave a good explanation of the difference between the prevention of infectious disease in the Renaissance (c1500–c1700) and the prevention of infectious disease in the eighteenth and nineteenth centuries (c1700–c1900) sometimes forgot to provide supporting knowledge and understanding from both periods (A01). In these cases, candidates also did not score full marks.

Most candidates stated that sweet smelling herbs or prayers were used as a prevention in the Renaissance period compared to the development of vaccinations in the eighteenth and nineteenth centuries. They explained this difference due to a change in the understanding of the cause of illness.

Examiners did note that this year more candidates were unable to explain or fully explain the difference between the preventions identified than in previous years. Therefore, they did not fully meet the level 2 criteria for A02. There were also a number of responses that stayed in Level 1 as they just stated a difference and were unable to provide evidence from both time periods. A number of other misconceptions on this question include candidates not understanding the difference between infectious disease and infection and mixing up prevention and treatment. Some candidates provided treatments as their supporting evidence which could not be rewarded.

While many candidates scored the full four marks, a few wrote far too much. Such answers demonstrated excellent knowledge but could not be rewarded beyond four marks and possibly the time taken here affected the completion of the longer answers which carried more marks.

3 Explain **one** way in which the prevention of infectious diseases in the Renaissance period (c1500–c1700) was **different** from the prevention of infectious diseases in the eighteenth and nineteenth centuries (c1700–c1900).

one way the Renaissance prevented infectious disease. Was the great plague. for example the would locked in their house for 40days with a guard ancared cross on the door. this lead to 15% of London being infected.

one way the prevention of infectious disease was different in the 1700-1900 was, for example the vaccination of cow pox. this lead to stopping of smallpox spreading. For ~~em~~ example those who had smallpox would be treated with cow pox.

ResultsPlus

Examiner Comments

The answer offers an example of a prevention used during the Renaissance and a prevention used in the eighteenth and nineteenth centuries. There is no explanation of change. Therefore, this response fulfils the requirement for a Level 1 answer. An answer displaying no qualities of A02 cannot be awarded more than the top of level 1.

ResultsPlus

Examiner Tip

Make sure that an explanation of difference (A02) is provided.

3 Explain **one** way in which the prevention of infectious diseases in the Renaissance period (c1500–c1700) was **different** from the prevention of infectious diseases in the eighteenth and nineteenth centuries (c1700–c1900).

In the Renaissance the prevention of infectious diseases was different because people still believed praying to God or bloodletting would prevent infectious diseases catching on from person to person. However, this is different in the Industrial Period since more logical explanations came into place like germs were the ~~cause of this people caught a disease, how~~ reason infectious diseases spread which led people into properly cleaning themselves and their homes to prevent catching the disease.

ResultsPlus

Examiner Comments

The answer offers a clear explanation of difference when it says "… more logical explanations came into place like germs". This meets the criteria for A02 Level 2. The candidate has supported this explanation of difference with specific examples for both the time periods in the question.

ResultsPlus

Examiner Tip

It is a good idea to state the similarity or difference clearly at the start of the answer.

Question 4

Most candidates had good knowledge and understanding of hospital care and treatment in the period c1700–c1900. Examiners felt that this question was fully accessible to all candidates and that there were very few Level 1 responses.

The best answers took an analytical approach rather than a descriptive approach to the question. Most candidates understood the needs to cover three aspects of content on this question. Most answers focused on Florence Nightingale and her drive for cleanliness and pavilion style hospitals, Joseph Lister and his development of antiseptics and James Simpson's development of chloroform as an anaesthetic. Some excellent answers were able to explicitly link developments, such as Florence Nightingale and cleanliness to a reduction in infection in hospitals, which lowered the death rate in hospitals. Some candidates were also able to explain that the developments made by Lister and Simpson improve surgery, a treatment available in hospitals, as more made people were willing to have surgery. When there was explicit focus on the question throughout the answer, candidates were able to achieve Level 4 for Assessment Objective 2 (analysis).

Common misconceptions on this question include a high portion of candidates writing a paragraph on Edward Jenner and the Smallpox vaccination or Medieval monastery hospitals as their third aspect of content. Some candidates mixed up antiseptics and anaesthetics. Several candidates wrote about the development of Germ Theory but were unable to explicitly link it back to improvements in hospital care and treatment in the period c1700–c1900. They often went on to reference treatments which were available in the 20th century and could not be rewarded.

A small proportion of candidates only provided two aspects of content and therefore their answers could not be deemed as wide ranging or precisely selected for Assessment Objective 1 (knowledge and understanding). Answers at Level 2 often described the work of Florence Nightingale and or Joseph Lister, without much development.

For answers that were awarded full marks, it was noticeable that many of these were relatively concise. These candidates had understood the focus on explaining causation and provided enough detail to support their explanation without becoming descriptive.

4 Explain why hospital care and treatment improved in the period c1700–c1900.

(12)

You **may** use the following in your answer:

- the Nightingale School for Nurses
- antiseptics

You **must** also use information of your own.

Hospital care and treatment improved in the period c1700 - c1900 due to the discovery of antiseptic surgery which got rid of all germs and ~~bacet~~ bacteria in the operating room during the surgery another way it improved was that Barbers need no longer surgeons as now to be a surgen you would have to go into education in order to get a degree to become a surgeon.

This is also supported by the Nightingale School For nurses as back then Nurses were working - days individually with no qualifications but now ~~they~~ you were required to

go to this school to learn how to became a nurse.

Another way it improved was due to the use of X-ray machines which better helped to detect an internal injury such as a broken bone without cutting some one open.

In conclusion Hospital are mostly improved in the c1900 due to multiple technological advancements which helped to improve Hospital care.

ResultsPlus

Examiner Comments

The answer reaches Level 2 for Assessment Objective 2 (analysis) as the link to the question is implicit. For Assessment Objective 1 (knowledge and understanding) the candidate also achieved Level 2, but it is lower in the level. The candidate has some knowledge of Lister, Nightingale and X-rays but these are undeveloped. Overall, this answer is a mid- Level 2 mark.

ResultsPlus

Examiner Tip

Make sure the details you include are used to support your analysis and explicitly linked to the question; don't just offer them as information.

4 Explain why hospital care and treatment improved in the period c1700–c1900.

(12)

You **may** use the following in your answer:

- the Nightingale School for Nurses
- antiseptics

You **must** also use information of your own.

Hospital care and treatment improved in the ~~pen~~ period c1700-c1900 as more was understood because of Florence Nightingale's School for Nurses. At the school, proper ways of caring and treating patients was taught, like keeping patients in seperate beds with distance between them or opening windows to allow air to flow through the rooms. This led to improved ~~bad~~ hospital care and treatment as many of these nurses went to hospitals and implimented these techniques, which caused death and injection rates in many hospitals to drop and people had a much better chance of living a hospital. This meant that hospital care and treatment improved as ~~the~~ patients were taken care of better and their treatment was more effective.

Another reason that hospitals care and treatment improved was the introduction of anesthetics. The first proper anesthetic to be introduced was chlorform, this could put patients to sleep ~~for a specific amount~~ and therefore surgeries and amputation could happen without the patient feeling any immediate pain. After chlorform other anesthetics were also introduced that could numb pain or

put people to sleep for longer. This meant that things like surgery were more viable as an option for patient as they did not have to go through all the pain. This led to improved ~~treatment~~ treatment as more advanced and longer surgeries could take place and the patient would then have a better ~~chace~~ chance of surviving, it also led to improved hospital care as patents syfering from pain or an illness could be given aesthetics to numb the pain.

A further reason why hospital care and treatment improved was the introduction of antiseptics. Antiseptics like carboxylic acid, ~~introduced~~ used/invented by Joesph Lister, would kill bacteria on someone body or tools and would reduce the chance of ingection from surgery or amputations. This meant that people were much more likely to survive surgery and ~~f~~ not be ingected, this along with aesthetics meant surgery became common treatment as it was no longer as dangerous as it once was. This led to better treatment as Surgery was more accsable and better hospital care as ~~pat~~ patients were far less likely to get ingected.

ResultsPlus

Examiner Comments

The answer reaches Level 4 for Assessment Objective 2 (analysis). It has a clear line of reasoning and an explicit link to the conceptual focus of the question at the end of each paragraph, which is sustained throughout the question. Assessment Objective 1 (knowledge and understanding) also reaches Level 4. There is accurate and relevant supporting knowledge, which is wide-ranging.
Overall, this answer resulted in a top Level 4 mark.

ResultsPlus

Examiner Tip

Make sure that each paragraph explicitly links to the question that has been asked.

Question 5

This was the more popular of the two option questions this year, and examiners noted candidates were confidently able to write about the causes of illness in the Medieval period (c1250–c1500). They felt that this question was accessible to all candidates.

Examiners noted that a substantial number of responses to this question were answered to a high standard. The best responses sustained their explanation throughout their answer (Assessment Objective 2). Candidates argued that the cause of illness in the medieval period was mainly based on religion due to the influence the Church had over society, miasma, the Theory of the Four Humours and astrology. The best responses were confident with the range of causes of illness and linked this to the preventions and treatments that were used as a consequence. Higher level responses were often able to judge that religion had the most significant effect on the cause of illness, as the Church prompted God as a cause of illness but also taught the Theory of the Four Humours, as it was linked to the ancient ideas of Hippocrates and Galen. This led many candidates to conclude that although there were different causes, as the main causes linked back to God, the cause of illness was based on religion in the medieval period.

A few candidates drifted into discussing ideas about medicine in general that were linked to the church, such as Galen's ideas about the anatomy rather than keeping a clear focus on the question. This often then meant for Assessment Objective 2, candidates' explanation became mainly directed at the question and placed into Level 3. Some candidates also provided information about the discoveries made by Vesalius and Harvey, but these could not be credited as they are outside the time period of the question and were not linked to the conceptual focus of the question (cause of illness).

Many knowledgeable answers remained at Level 3 as candidates were unable to sustain their link towards the question explicitly, which then led to their judgement having only some justification. Answers at Level 2 often described ideas, such as the Theory of the Four Humours and or the alignment of the planets. At Level 2 candidates often implied the link to religion being the main cause of illness for Assessment Objective 2.

Examiners also noted that the majority of answers were well structured, with candidates trying to provide a minimum of three aspects of content to answer the question.

Chosen question number: **Question 5** ☒ **Question 6** ☒

I partially agree with the statement as many inaividuals blamed cause of illness on God, as a punishment to them for sinning. This led to the sick praying for forgiveness and even whipping themselves, also known as self-flagilation just to prove their guilt× in their sins.

However, I could also disagree with the fact that Miasma was also strongly ~~agreed~~ belived as many thought bad smells in the air was making them sick. This was highly belived so many inaividuals had pomanders and herbs to smell so that they don't take in the bad fumes in the air.

I could disagree again with the fact that the idea Hippocrates made about the 4 humours was belived the strongest in the medival period. This was a theory that the humours could imbalance and make people ill. The treatment to this was purging ~~if yellow bile~~, blood letting, and then eating something cold if you are warm. The 4 humours, also known as black bile, yellow bile, blood and phlegem was belived by most of the population.

ResultsPlus

Examiner Comments

This response meets the demands of the mark scheme for Assessment Objective 2 (analysis) at Level 2. There is limited analysis throughout the answer with only implicit links to the question. The response gives some relevant knowledge and understanding linked to three aspects of content and therefore meets the demands for Assessment Object 1 (knowledge and understanding) at Level 2. The judgement is stated at the beginning of the answer but is insecure. Overall, this is a Level 2 response.

ResultsPlus

Examiner Tip

Try to explicitly link each paragraph to the conceptual focus of the question.

Chosen question number: **Question 5** ☒ **Question 6** ☒

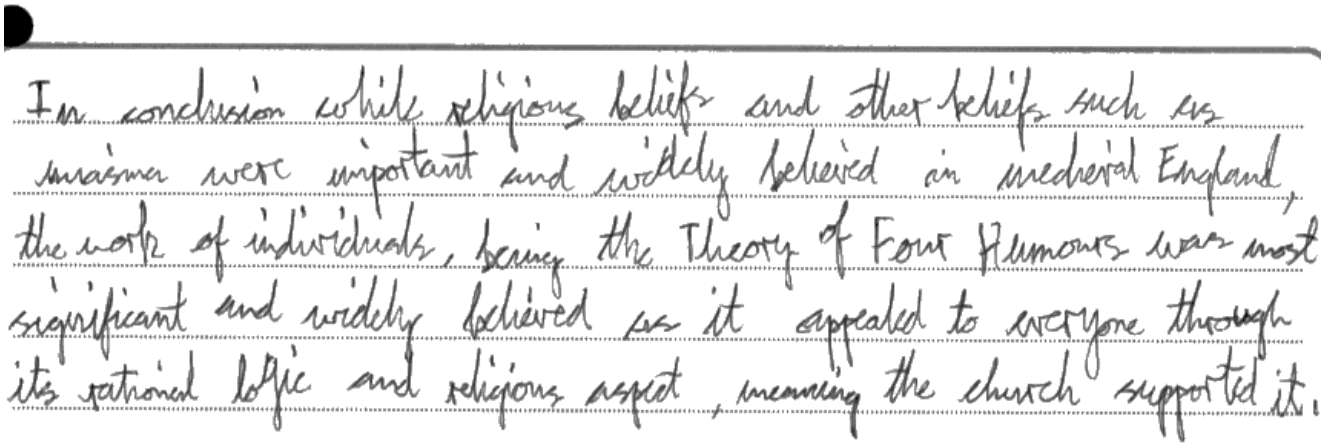
I slightly ~~agree~~ disagree with this argument. While religion was a significant factor in beliefs about the cause of illness, the rational beliefs of individuals such as Galen were more significant in the medieval period.

The church was the main authority in England during the medieval period. They believed that illness was a punishment, sent by God from sin. This belief was known as divine retribution. It was a prominent belief among society in England who were devoutly Christian, particularly during the Black death in 1348, where it was believed to be the most likely cause. It inspired preventative measures such as prayer and self-flagellation to atone for sins in the hope that they wouldn't become infected. While it was believed widely, it was drawn back by the fact it wasn't at all rational, leading some people such as physicians to believe in other causes. Therefore, religion was a significant factor in beliefs about cause of illness, it wasn't rational so some chose to believe in other ideas about causes.

Another prominent belief was the idea that bad air caused illness; this was known as miasma. It was another belief for the cause of the Black Death and inspired preventions such as pomanders, sweet smelling charms, worn

to ward off bad air. While it was prominent, it wasn't as widely believed as other ideas as the church didn't believe it and support it; to them it didn't prove that illness was a punishment from God, so it didn't benefit them for people to believe in miasma. As a result, while it was a prominent belief in medieval England, as the church didn't support it, it wasn't as widely believed.

Arguably the most significant and widely believed cause of illness was humourism and the theory of opposites. Initially theorised by Hippocrates, the body was thought to have four 'humours': Black bile, Yellow bile, phlegm and blood. The idea was that an imbalance in these humours caused illness. Galen developed this with the theory of opposites. This proposed treatments to regain balance of humours and treat illness. For example blood letting was a widely practiced treatment to remove excess blood. This idea was so widely believed as it was rational, meaning it had logic and made sense. Not only that, the church supported it as the believed it proved the existence of a divine creator (God) as the body was very complex. Therefore by appealing to physicians and the wealthy religious public, the work of individuals through the theory of Four Humours became the most significant and widely believed idea concerning the cause of illness.



ResultsPlus

Examiner Comments

This answer meets the demands of the mark scheme for Assessment Objective 2 (analysis) at Level 4. The answer offers a line of reasoning throughout which is consistently directed at the conceptual focus of the question. The analysis is supported by wide-ranging and precisely selected knowledge. Therefore, for Assessment Objective 1 (knowledge and understanding) this answer reaches level 4. There is a clear judgement reached so again meets the requirements for Level 4. Overall, this is a high-Level 4 response.

ResultsPlus

Examiner Tip

The best answers have a sense of evaluation running throughout the analysis.

Question 6

This was a less popular question with candidates. Although less popular, examiners felt that the question was still accessible to all candidates. The focus of the question was on the role of science and technology as the most important reason why medicine progressed in the modern period (c1900–present).

The best responses sustained their explanation throughout their answer (Assessment Objective 2). The majority of candidates supported the factor named in the question by drawing on a wide range of examples from modern medicine. Commonly used evidence included developments such as X-rays, MRI scanners and keyhole surgery. Stronger candidates were also able to argue that science and technology was significant as it led to the discovery of Salvarsan 606. Candidates explained how Salvarsan 606 was used to treat syphilis and represented a significant breakthrough in targeted treatments. Many candidates also explored the role of science in the identification of DNA by James Watson and Francis Crick. Stronger responses linked this development to the Human Genome Project and its impact on the diagnosis of illness and its implications for the treatment of disease. To counter their argument candidates often considered the role of government. Common examples included the creation of the National Health Service in 1948, which improved access to healthcare and contributed to improvements in public health with the government-led public health initiatives, such as the Change4Life campaign and the smoking ban introduced in 2007. Candidates also focused on the role of individuals, such as Alexander Fleming's discover of penicillin.

Again, many knowledgeable answers remained at Level 3, as they were unable to consistently focus on the conceptual focus of the question. Answers at Level 2 often briefly developed some examples of developments in medicine such as the NHS, penicillin, or types of surgery, rather than developing the explanation of how these contributed to progress in medicine. These responses often scored Level 2 for Assessment Objective 2 (A02) as their analysis was implied towards the question. Some candidates also focused on the foundations of the NHS without a link to medical progress.

Examiners also noted a common misconception lay in the understanding of time periods. Candidates included developments from the Industrial period such as Florence Nightingale or the development of anaesthetics and antiseptics. Although the knowledge was accurate it could not be rewarded as it is outside the time period of the question.

Chosen question number: **Question 5** ☒ **Question 6** ☒

I totally agree that Science and technology is the main reason why there has been progress in medicine in the modern period (1900-present). This is because I think this period has the greatest amount of medical knowledge out of any other. Through the technology and past discoveries of surgery it now has an almost guaranteed success rate. The improvement in Science and technology has created health sectors specifically for certain parts of the body and illnesses/diseases. This means it is much easier to be healed of a health risk now than ever. ~~Through~~ The first ever ~~anti~~ the antibiotic discovered in 1924 by Alexander Fleming allowed for disease and illness to be stopped easily.

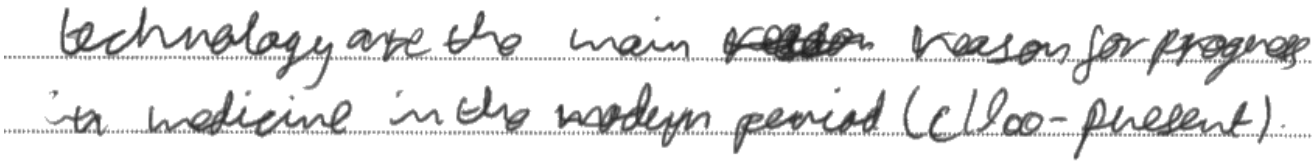
Technology & Science also allows for the NHS to provide Free health care through the public health aid and taxes, meaning people get ~~free~~ specialised high-tech treatment for pretty much free. Technology and Science allows for injuries, illnesses, ~~diseases~~ all to be diagnosed without even having to look inside the body. Through

X-rays and blood tests you can have a diagnosis quickly, effectively and with little to no change of side effects. In the 20th century huge discoveries like the first magic bullet, Stephenson-166, and the discovery of DNa (1953), have allowed us to not just treat somebody, hoping it works, but to do it knowing it will.

Not only has treatment gotten better but general public health knowledge has too. Through modern technology people have a better understanding of health and it's also shown through campaigns, charities, and adverts. For example anti-smoking or second hand smoking.

The creation of prosthetic limbs and transplants has allowed for life changing procedures meaning people can continue to live happy normal lives if born without an arm for example. Heart pacemakers and hearing aids are another example as they do manually do/have job with little to no constant supervision.

To conclude I totally agree with the statement improvements in Science and



ResultsPlus

Examiner Comments

This answer meets the demands of the mark scheme for Assessment Objective 2 (analysis) at Level 2. There is explanation but the link to the question is implied. For Assessment Objective 1 (knowledge and understanding) this answer reaches level 3 as there is good knowledge and understanding. An overall judgement is Level 2 as it is insecure. Overall, this is a low Level 3 response.

ResultsPlus

Examiner Tip

Check the focus of the question as well as the topic, so that you can make sure the detail you include is relevant.

Chosen question number: **Question 5** ☒ **Question 6** ☒

I agree with the idea that ~~[illegible]~~ improvements in technology and science was the main reason behind progress in medicine due to the developments of medical scans. Wilhelm Röntgen, a German scientist discovered and developed the properties of x-rays in the year 1895 - some historians may argue this had a profound impact on the improvement of medicine in c1900-present due to their continual use in diagnosis of broken or damaged hard tissues within the body. Even though invented in the late 1800's, the X-ray was frequently used throughout the 1900's particularly during the War. American scientist William Coolidge developed the X-ray further via the "Coolidge tube" which reduced the effects of overheating in 1913. Later, the X-ray was developed into a computer tomography (CT) scan by Godfrey Hansfield in the 1900's and in the same year, the first MRI (Magnetic resonance imaging) scan was used to diagnose patients. These developments in medical scans has helped doctors understand more about what is behind the cause of disease and how and what places within the body need to be repaired or treated, reducing the need for invasive surgery, therefore ~~[illegible]~~ lowering the risk of infection.

Therefore, I agree with the idea that science and technology were the main factors behind the progress in Medicine c1900 - present day due to their help in diagnosis & the spread of disease or further damage to internal organs and tissue can be prevented.

A second reason why I agree with the idea that science and technology were the most important factors in the improvement and progress in medicine c1900 - present day was due to the production of magic bullets. Above all, science would be the primary factors behind the discovery of the first magic bullets. Magic bullets are synthetic antibodies that target and destroy specific diseases. After the bacteria that caused STI, syphilis was discovered in 1905, Paul Ehrlich set at, trying different arsenic compands in an attempt to find a magic bullet that could treat syphilis. In 1909, Sahachiro Hata joined Ehrlichs team and discovered that of the many arsenic compounds that had failed to cure the bacterial infection (over 600), the 606'th compand seemed to work; the first magic bullet had been found. Two years later in 1911, the magic bullet was first tested on a human under the trade name Salvarsan 606. Later, in 1932, Gehrard Domagk sought

to find a magic bullet that could cure streptococci, a bacteria causing blood poisoning, primarily because his daughter was suffering from blood poisoning herself. In 1932, Domagk successfully found a magic bullet that could cure his daughter and tested it on her - she survived and recovered. Domagk had found the second magic bullet: Prontosil. Therefore, I believe that the role of science - particularly in finding synthetic antibodies to cure disease - was one of the main factors why medicine progressed in c1900 - present day.

However, some historians believe that government lifestyle campaigns and government legislation was the main reason medicine has progressed in c1900 - present day. For example, in 2004, the government launched the Change 4 Life campaign that promoted healthy lifestyle and healthy food consumption choices. This has led people to want to live a more healthy lifestyle in order to access the benefits of living healthily. Secondly, 5 years prior to this, in 2004, the government launched the DrinkAware campaign that promoted the moderation of drinking alcohol and the narrowed consumption by raising awareness of the risk factors it has towards liver and kidney cancer. Government legislation has also played an important part in the progress of Medicine too. For example

after the Great Smog of 1952, the guernment ~~[illegible]~~ passed the Clean Air Act of 1956 - and a later Clean Air act in the 1980s. As an attempt to prevent the amount of smokes in Britain after recognising the damage it can cause to the lungs in the body. the guernent banned all cigarette advertising in 2000, banned smoking in public places in 2007 and placed a ban on smoking in vehicles ~~[illegible]~~ containing passengers in 2015. These safety measures could be held accountable for being the main reason why medicine has progressed in c1900- Present day as they have rapidly decreased the number of people affected with self-inflicted diseases such as kidney and lung cancer

Overall, I believe that eventhough guernment campaigns and legislation has lead to a rapid improvement in health in Britain, the medicinical technological advancements of the 20th century and the role of science has had a more profound impact on diagnosis and treatment of disease. Goverment legislation helped prevent disease, but the avid technologies and scientific procedures can cure and prevent diseases which is why I believe they have had a more significant impact on the progress of medicine c1900- present day.

ResultsPlus

Examiner Comments

This answer meets the demands of the mark scheme for Assessment Objective 2 (analysis) at Level 4. The answer offers a line of reasoning throughout which is consistently directed at the conceptual focus of the question. The analysis is supported by wide-ranging and precisely selected knowledge. Therefore, for Assessment Objective 1 (knowledge and understanding) this answer reaches level 4. There is a clear judgement reached so again meets the requirements for Level 4. Overall, this is a high-Level 4 response.

ResultsPlus

Examiner Tip

The best answers have a sense of evaluation running throughout the analysis.

Paper Summary

Examiners commented that there was a number of impressive answers where candidates seemed well-prepared and demonstrated excellent knowledge being deployed to support thoughtful analysis and evaluation. In particular, candidates seemed well prepared for the 8 mark source question and the 12 and 16 mark questions, with most answers having a clear structure and good use of specialist terms.

If extra paper is taken, candidates should state clearly in the answer space for the question that it has been continued on separate paper and this should be clearly labelled. However, in many cases where additional paper had been taken, the marks had already been attained within the space provided rather than on the extra paper and candidates should be discouraged from assuming that lengthy answers will automatically score highly. Indeed, candidates taking extra paper sometimes ran out of time on the final, high mark question and therefore disadvantaged themselves.

Examiners reported that a poor standard of handwriting made a number of answers difficult to mark and exacerbated the difficulty in understanding a badly-expressed answer. Also, a failure to structure answers in paragraphs made it difficult for the examiner to identify a line of reasoning and to check whether three different aspects have been covered.

Where there has been weaker performance, the following points can be made:

- Candidates need a secure understanding of the chronological periods and terms used in the specification as well as how the term 'century' is used, for example the nineteenth century refers to dates within the period 1800–1899.

· Candidates need to understand the themes within the specification such as the cause of illness, prevention of illness, treatment of illness or access to care.

- Candidates need to understand the factors within the specification such as the role of the government, individuals, science and technology and attitudes within society.
- A number of knowledgeable answers failed to reach the highest level because they were not focused on the specific question being asked or did not deploy precise detail to support the analysis.
- It is not necessary to use the question's stimulus points and candidates should not attempt to do so if they do not recognise them; however, candidates should aim to cover three aspects of content.
- While there was good knowledge of some topics, candidates cannot rely on knowing just a few key topics and hoping to use that information whatever question is asked.

Grade boundaries

Grade boundaries for this, and all other papers, can be found on the website on this link:

https://qualifications.pearson.com/en/support/support-topics/results-certification/grade-boundaries.html

Pearson Education Limited. Registered company number 872828
with its registered office at 80 Strand, London WC2R 0RL.